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GUIDELINE FOR PHASE 2-4 CLINICAL TRIAL PARTICIPANT TIME, INCONVENIENCE AND EXPENSE (TIE) COMPENSATION MODEL

This document has been prepared to serve as guidance to applicants/sponsors of clinical trials during the use of registered or unregistered medicines in approved clinical trials. It represents the South African Health Products Regulatory Authority's (SAHPRA) current thinking on participant reimbursement during clinical trial participation and proposes a minimum compensation that can be paid. It is not intended as an exclusive approach and SAHPRA reserves the right to request any additional information and may make amendments in keeping with the knowledge that is current at the time.

Document History

Final Version	Reason for Amendment	Effective Date [dd Month yyyy]
1	First version published for implementation	June 2018
2	- Section 2, reimbursement amount (inflation consideration) - Old document number 2.51 changed to SAHPGL-CEM-CT-02	July 2022
3	Added information to Section 2: - Inflationary adjustments to recommended amounts Participant remuneration amounts have been updated to reflect inflationary increases since the initial budget approval. The revised rates are intended to maintain equivalent real value for participants' time, travel, and inconvenience. - Clarifications on existing remuneration components To ensure consistency across sites and clarity for both participants and site staff, the following points have been clarified: - Remuneration applies to all completed visits and activities - Remuneration for participants who screen fail - Payments will be made via EFT, voucher, or cash, and for this reason, amounts will be rounded up to facilitate this payment - Additional Remunerated Activities Following recent changes in study conduct, several new participant activities now qualify for remuneration. These include: - Reimbursement for telephonic visits - Remuneration for unscheduled visits - Participation in any remote data collection activities (for example, paper/digital diary completion or device-based monitoring)	01 June 2026

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Glossary

Abbreviation/ Term	Meaning
AA	Automobile Association of South Africa
CIOMS	Council for International Organizations of Medical Sciences
eCOA	Electronic Clinical Outcome Assessment
NHREC	National Health Research Ethics Council
REC	Research Ethics Committees
SAHPRA	South African Health Products Regulatory Authority
TIE	Time, Inconvenience, and Expenses
UNAIDS	United Nations Programme on HIV/AIDS

1. BACKGROUND

As per the published National Health Research Ethics Council (NHREC) guidelines (2024) titled “South African Ethics in Health Research Guidelines: Principles, Processes and Structures,” Section 3.1.8 states that trial participants should be compensated appropriately for their time (T), inconvenience (I), and expenses (E)¹. In addition, the Council for International Organisations of Medical Sciences (CIOMS) Guideline states that participants may be compensated for inconvenience and time (CIOMS, 2016)². United Nations Programme on HIV/AIDS (UNAIDS)(2012) notes that participants should be compensated for expenses related to participation, time, and inconvenience³.

The South African Health Products Regulatory Authority (SAHPRA) has considered the above and adopted the TIE compensation of clinical study participants in South Africa.

2. PURPOSE AND SCOPE

This document guides Applicants/Sponsors/Investigators conducting studies and applies to Phase 2 to 4 studies conducted in South Africa. Phase 1 studies include a higher risk for participants, hence they should be compensated on a different scale following Phase 1/First in human remuneration guidelines.

3. LEGAL PROVISION

Guideline is established in terms of Regulation 30 of the Medicines and Related Substances Act, 101 of 1965, for the conduct of clinical trials.

4. MODEL FOR COMPENSATION FOR CLINICAL STUDY-RELATED VISITS

4.1 Time spent

Current minimum wages in South Africa for unskilled labour start from approximately R25-R30 per hour⁴. Factoring in the inconvenience aspect, R50 per hour is recommended⁴.

Note:

- 4.1.1 Amounts within a few rands of more cash-manageable numbers may be rounded up, e.g., R99 to R100. This is in order to ease the burden of unnecessary calculation and difficulty in petty-cash flow at sites.
- 4.1.2 In general, a standard scheduled visit should not take longer than 3 hours. More intensive visits may, however, require additional time at the site. This would necessitate a higher “compensation” as the participant is now spending an extended period of time away, resulting in them missing the majority of their work/school day as opposed to a portion of it. In such a case, this amount should be increased to R50 per hour.

4.2 Inconvenience (considered in the light of travel/distance from home to a site)

Based on rates prescribed by Automobile Association (AA) of South Africa⁵ and Stellenbosch University⁶, it is recommended that R4.75 per km travelled is paid. The travel cost is broken down into 3 categories (0-25, 26-50, and >50 km). For example, a participant falling in the first category will receive R237.50 travel compensation ($R4.75 \times 25 \text{ km} \times 2$ [return trip]). This should be rounded up to R250 for convenience and to make provision for any parking costs.

Note:

- 4.2.1 It would be ideal for each site Principal Investigator to decide on the radius around the trial site

that maximum recruitment efforts would be focused on. This would allow for a standardised approach to be adopted per site and per study, leading to less per participant variability of travel-related costs. When participants are identified beyond the catchment area identified or move beyond the predefined catchment, additional compensation could be explored on a case-by-case basis.

4.3 Expenses

It is recommended that all participants be offered a meal to the value of R100. If this amount is included in the ICF reimbursement total, then the R100 should be removed from the total, documented in the relevant documentation and explained to the participant prior to this adjustment being made. Should the participant be at the site for longer than 3 hours, an additional meal or money to the value of R100 should be made available to them.

Table 1: Compensation Model for a Standard and Extended Clinic Visit

TIME DISTANCE	Standard visit 2-3 hours	Extended visit >3 hours
0-25 km	Travel = R250 (Return trip [rounded up]) Inconvenience = R150 (R50 x 3h [rounded up]) Expenses = R100 (meal & refreshment)	T = R250 I = R150 + R50 per hour (over & above 3h) E = R200 (2 x meals & refreshment)
	Total = R500	Total = R600 + R50 per hour
26-50 km	T = R500 (Return trip [rounded up]) I = R150 E = R100	T = R500 I = R150 + R50 per hour (over & above 3 h) E = R200 (2 x meals & refreshment)
	Total = R750	Total = R850 + R50 per hour
>50 km	T = R500 + R4.75 x km (over & above 50 km) I = R150 E = R100	T = R500 + R4.75 per km (over & above 50 km) I = R150 + R50 x h E = R200
	Total = R750 + R4.75 per km	Total = R850 + R50 per hour + R4.75 per km

Note: SAHPRA is of the view that in 2026, a minimum of R500 would be the most appropriate compensation level for a standard participant's visit. SAHPRA may review this model periodically, as necessary, taking into account economic conditions and regulatory developments.

4.4 Additional Inconvenience

Some studies call for more invasive procedures (e.g., PK sampling) or strenuous procedures (e.g., Stress ECGs), which are over and above the standard inconveniences of a study visit. Therefore, a case can be made for additional compensation. Other inconveniences which are not always so apparent need to also be compensated for, such as an HIV trial in which participants are required to abstain from sexual activity and product use for specified periods. These additional procedures must be described in the Protocol and

Informed Consent Form, the additional remuneration specified, and justification of these amounts provided. However, if on the visit day the participant reports not having adhered to these procedures, the specified amount should be deducted from the overall ICF amount, documented and explained to the participant.

4.5 Scheduled telephonic visits

Telephonic visits are generally conducted in one of the following scenarios:

- The protocol specifies that a scheduled visit be conducted telephonically as opposed to onsite, or
- A volunteer is unable to attend the scheduled visit for a specific reason, and the site is instructed by the protocol and/or Sponsor to conduct the visit telephonically.

This is still considered to be a trial-related activity, and the participant should thus be remunerated for this task. A minimum amount of R150 is recommended.

4.6 Home-based/virtual consultation visits

When permitted by the protocol, a scheduled visit may be conducted either at home or virtually. The participants must be remunerated for time and inconvenience. A minimum of R200 per visit is recommended; this could be increased depending on the time and complexity of the visit.

4.7 Unscheduled visits

Unscheduled visits may occur on site, telephonically, virtually, or at home. The same remuneration as detailed above applies.

4.8 Screen Failures

A screen failure occurs when a potential volunteer is invited to the site, signs an informed consent, and undergoes at least part of a screening visit (such as blood tests, ECG, and/or medical history review), but ultimately does not meet the inclusion/exclusion criteria and therefore does not proceed to randomization or treatment.

- 4.8.1 When the screening visit has been concluded in its entirety, the full remuneration is due to the participant.
- 4.8.2 When the screen failure is identified before all the screening procedures have been completed and the patient has not necessarily been allocated a screening number, partial reimbursement for the visit is expected. This should be 20-50% of the total reimbursement.

4.9 Parent/ Legal guardian/ caregiver

In studies where the participant requires the presence of a parent/legal guardian/caregiver, the same Time, Inconvenience and Expense (TIE) remuneration framework shall apply. Reimbursement should reflect the additional time, inconvenience and reasonable expenses incurred by the accompanying individual, while remaining aligned with ethical principles that prohibit undue inducement. Where both the participant and the parent/legal guardian/caregiver incur separate time and travel burdens, the reimbursement structure must transparently account for this and be clearly justified in the study budget for REC approval.

4.10 In-patients

In-patients recruited to the trial need not be compensated for distance travelled, but the inconvenience, expense, and time beyond in-patient stay need to be taken into consideration. A minimum of R100 to R250 per visit is recommended, depending on whether informed consent or other investigations and/or patient-

reported outcome questionnaires are being done.

4.11 Patient diary completion

The completion of paper and/or electronic diaries between visits is increasingly being incorporated into clinical trials. This amounts to an additional trial-related activity and can be inconvenient and burdensome for trial participants to complete these diaries. The participant should thus be remunerated for this task. A minimum amount of R100 to R150 per completed diary entry is recommended, depending on the extent of the trial-related procedures (e.g. dosing, temperature measurements, etc.) that need to be captured.

In addition, when using electronic diaries, it must be highlighted that most people in South Africa do not have access to free wi-fi. Free wi-fi and/or sufficient airtime should be made available to all participants to complete the necessary trial-related activities. Alternatively, sponsors can provide devices that have data preloaded.

4.12 Electronic Clinical Outcome Assessment (eCOA), including the use of tablets, smartphones, wearables, and other such devices

As for 4.11 above, participants need to be remunerated appropriately for this task. In addition, sufficient airtime needs to be made available to all participants, and/or a device with pre-loaded data needs to be provided by the Sponsor.

5. REFERENCES

The following related documents are referenced:

1. National Department of Health. 2024. South African Ethics in Health Research Guidelines: Principles, Processes and Structures.
2. Council for International Organizations of Medical Sciences (CIOMS). 2016. International Ethical Guidelines for Health-related Research Involving Human Subjects.
3. United Nations Programme on HIV/AIDS (UNAIDS). 2012. Ethical considerations in Biomedical HIV prevention trials.
4. National Minimum Wage Act 9 of 2018.
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5. Prescribed rate per kilometer: <https://accountingacademy.co.za/news/read/new-sars-tax-free-rate-per-km-1> Accessed 15 February 2025.
6. Vehicle Services. 2022.
<https://www0.sun.ac.za/voertuigvloot/media/pdfs-docs/2022/VEHICLE%20POOL%20KM%20and%20DAY%20tariffs%20since%201%20Jan%202022.pdf>. Accessed on 15 February 2022.

6. VALIDITY

This guideline is valid for a period of 5 years from the effective date of revision and replaces Guideline for Clinical Trial Participant Time, Inconvenience and Expense (TIE) Compensation Model, old document number 2.51. It will be reviewed on this timeframe or as and when required.